INFORMATION TO KEEP WITH YOU

My complete treatment handbook

FLOX: fluorouracil, folinic acid and oxaliplatin

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
56 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00486
Version 8

Fluorouracil and folinic acid on Days 1, 8, 15, 22, 29 and 36; oxaliplatin also on Days 1, 15 and 29. Two weeks off follow. Three 56-day cycles make a planned 24-week course.

Treatment stages

01 **Prescribed treatment course**
Adjuvant stage II or III colon cancer after resection

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

FLOX: fluorouracil, folinic acid and oxaliplatin · National Cancer Control Programme (NCCP) 00486

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for fever, shivering, sudden illness, severe diarrhoea, inability to drink, very little urine or unusual bleeding. New severe chest pain, collapse or major breathing difficulty needs emergency help.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and what they do

FLOX is given after surgery for selected stage II or III colon cancer to reduce the chance of recurrence. It combines weekly fluorouracil/folinic acid with oxaliplatin on alternate treatment weeks.

PLATINUM CHEMOTHERAPY

Oxaliplatin

Eloxatin



Links deoxyribonucleic acid (the cell's genetic material) (DNA) strands

Oxaliplatin makes links within DNA. These interfere with copying and repair, which can stop a cancer cell dividing. Healthy tissues can also be affected.

Platinum enters cell → DNA links form → Copying is disrupted

What to watch for

Cold can trigger tingling or throat discomfort. Tell the team about persistent numbness, difficulty with buttons or walking, or infusion symptoms. New breathing difficulty always needs assessment.

CHEMOTHERAPY MODULATOR

Folinic acid

Calcium leucovorin



Strengthens fluorouracil action

Folinic acid helps fluorouracil remain bound to an enzyme needed to make DNA. In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

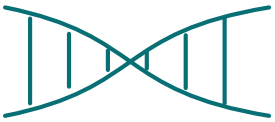
What to watch for

The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with DNA and RNA, the instructions cells need to grow. The same drug has different schedules; follow the prescription for this regimen.

Drug enters cell → DNA / RNA disrupted → Growth slows

What to watch for

Report diarrhoea, mouth sores, painful hands or feet and unexpected severe early symptoms. Chest pain needs urgent assessment even after a normal dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) test.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim and duration

Your course

Fluorouracil and folinic acid on Days 1, 8, 15, 22, 29 and 36; oxaliplatin also on Days 1, 15 and 29. Two weeks off follow. Three 56-day cycles make a planned 24-week course.

Choices

Discuss the expected benefit, burdens, alternatives and how treatment fits your priorities. You can ask for more time or another explanation before deciding.

Understanding the evidence

Ask which evidence matches your treatment aim.

NSABP C-07 tested adding oxaliplatin to weekly bolus fluorouracil/leucovorin after stage II/III colon cancer surgery. FLOX is not the 46-hour-pump FOLFOX regimen. Group results do not predict your individual benefit.

SACTCHECK · CHAPTER 02

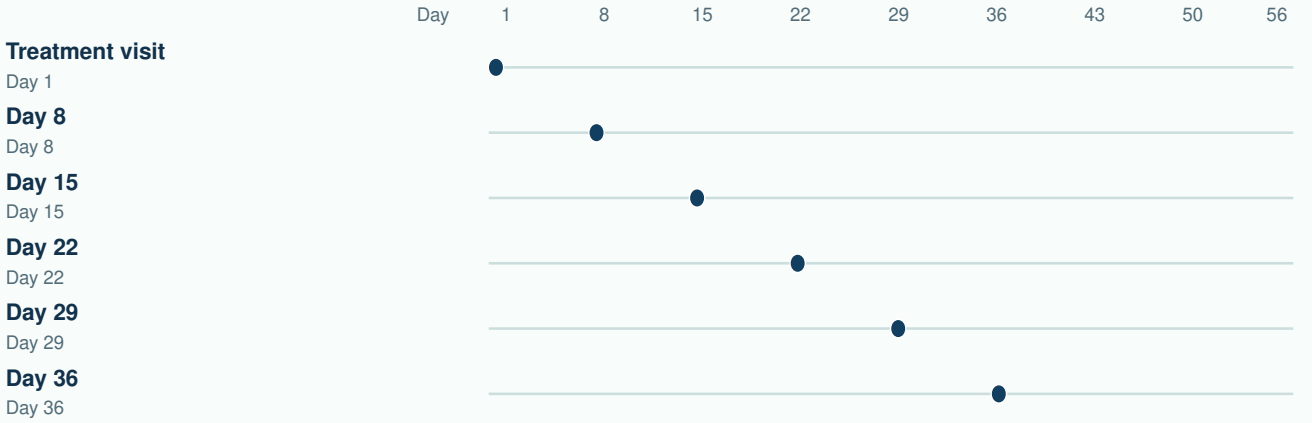
Your schedule, tests and appointments

FLOX: fluorouracil, folinic acid and oxaliplatin · NCCP 00486

Your treatment plan

FLOX: fluorouracil, folinic acid and oxaliplatin

56-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ADJUVANT STAGE II OR III COLON CANCER AFTER RESECTION

Prescribed treatment course

Start | Treatment | Review

Fluorouracil and folinic acid on Days 1, 8, 15, 22, 29 and 36; oxaliplatin also on Days 1, 15 and 29. Two weeks off follow. Three 56-day cycles make a planned 24-week course.

The treating team confirms the indication, schedule and duration. Changes are recorded on your own prescription.

BASELINE → TREATMENT → FOLLOW-UP

Tests and checks

Required safety blood tests are separate from the individually prescribed scan or tumour-marker plan.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00486 v8 official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

FLOX: fluorouracil, folinic acid and oxaliplatin: treatment and checks

Adjuvant stage II or III colon cancer after resection

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Review	After treatment
Blood count / kidneys / liver NCCP 00486	Baseline tests and DPD status	Before each weekly treatment	Extra tests if unwell or dehydrated	Results guide any delay or change	Follow unresolved abnormalities
Symptoms and medicines NCCP 00486	Review medicines, cardiac history and usual function	Review bowel, mouth, skin and infection symptoms	Use urgent contact plan	Confirm recovery and next dose	Agree recovery care
Scans / tumour markers NCCP 00486	Confirm disease assessment	According to individual plan	New symptoms may bring review forward	Consultant-directed response assessment	Individual follow-up
Nerves / heart NCCP 00486	Record neuropathy; electrocardiogram (ECG) if compromised cardiac function	Report persistent neuropathy before doses; formal neuropathy review every 2 cycles	New chest pain or breathlessness: urgent assessment	ECG as clinically indicated	Nerve symptoms can persist after treatment

What each check is for

1 **Confirm your personal plan**

Before starting

FLOX is given after surgery for selected stage II or III colon cancer to reduce the chance of recurrence. It combines weekly fluorouracil/folinic acid with oxaliplatin on alternate treatment weeks.

Ask / **act:** Agree medicines, treatment days, duration and monitoring.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before treatment

Confirm the treatment aim, duration, DPD result, blood tests, prescription and contact numbers.

STEP 2

Visits and access

Fluorouracil and folinic acid on Days 1, 8, 15, 22, 29 and 36; oxaliplatin also on Days 1, 15 and 29. Two weeks off follow. Three 56-day cycles make a planned 24-week course.This regimen does not require a home infusion pump.

STEP 3

If treatment is delayed

Ask for revised written treatment and support-medicine instructions. Do not make up missed doses or carry an old plan into a new phase.

STEP 4

Getting help

Keep the wallet card and medicine list available. Contact the team promptly for infection, dehydration, chest symptoms or significant toxicity.

Tests and safety

DPD before fluorouracil

Low DPD activity can cause severe toxicity. Testing helps guide the plan but cannot prevent every reaction. Unexpected severe early symptoms need urgent advice.



Blood tests

Before each weekly treatment: blood count, renal and liver profile; baseline DPD. Scans/markers follow the consultant plan.



Medicines and interactions

Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.

Symptoms and supportive care

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Day 1 → Day 56: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment visit	 **Day 1** All three medicines
Day 8	 **Day 8** Fluorouracil + folinic acid
Day 15	 **Day 15** All three medicines
Day 22	 **Day 22** Fluorouracil + folinic acid
Day 29	 **Day 29** All three medicines
Day 36	 **Day 36** Fluorouracil + folinic acid
Planned rest	 **Days 43–56** No catch-up doses; symptoms may persist

The written plan comes first: Fluorouracil and folinic acid on Days 1, 8, 15, 22, 29 and 36; oxaliplatin also on Days 1, 15 and 29. Two weeks off follow. Three 56-day cycles make a planned 24-week course.

Symptoms can continue: Contact oncology for fever, sudden illness, diarrhoea, mouth soreness or inability to drink, during treatment or the break.

Why these bands stay broad: Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00486 v8 official regimen ↗

There are no guaranteed "safe days". Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Between-treatment notes

Day 1–42: Six weekly treatment visits

Attend on Days 1, 8, 15, 22, 29 and 36; oxaliplatin only on Days 1, 15 and 29. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Report symptoms before the next dose; do not wait for your next appointment if urgent.

Day 43–56: Planned two-week break

No catch-up doses; symptoms can continue during the break. Confirm the next plan. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Report symptoms before the next dose; do not wait for your next appointment if urgent.

Support around the medicines

1 **Supportive medicines**

Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.

2 **Mouth, bowel and skin care**

Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.

3 **Medicine checks**

4 **Cold precautions**

Avoid cold exposure as advised after oxaliplatin. Do not suck ice chips during this regimen.

Recognise symptoms and act

Mouth / eating

Report painful ulcers or difficulty eating and drinking.

Chest / heart

New chest pain needs urgent assessment, including during an infusion.

Bowel / hydration

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

Hands / feet

Report painful redness, cracking or peeling that affects daily activity.



Blood / infection

Fever, shivers or sudden illness need immediate oncology advice.

Energy / daily life

Report fatigue that limits usual activity; ask for practical and nutrition support.

Nerves

Cold can trigger tingling. Report lasting numbness or difficulty with buttons or walking.

Throat / breathing

Report throat tightness or breathing difficulty promptly; the team must assess the cause.

Daily life and support

Food and fluids

Aim for tolerable meals and fluids. Report weight loss or poor intake; ask for dietetic support.

Activity and rest

Adjust activity and rest to your energy. Report fatigue that limits normal tasks.

Fertility and sexual health

Ask about fertility preservation before treatment and the contraception and breastfeeding plan for all medicines. Report a possible pregnancy promptly.

Support for you and your family

Ask about counselling, psycho-oncology, practical support, transport and financial advice. Bring someone to discussions if helpful.

Questions worth asking

Which treatment plan applies to me?

Fluorouracil and folinic acid on Days 1, 8, 15, 22, 29 and 36; oxaliplatin also on Days 1, 15 and 29. Two weeks off follow. Three 56-day cycles make a planned 24-week course. Your team confirms any modification.

Do more side effects mean treatment is working?

No. Side effects do not reliably measure response. Report symptoms and follow the agreed review plan.

Who do I contact overnight?

Ask the team to replace example numbers with working daytime, out-of-hours and backup contact routes before leaving.

Try explaining the plan back: Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 1, 15 and 29: oxaliplatin + fluorouracil/folinic acid

Active medicines: Oxaliplatin + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally once	Before the oxaliplatin-containing dose Review QT risk, interactions, constipation and hepatic function.
Dexamethasone	8 mg orally once	Before the oxaliplatin-containing dose Reconcile all steroid prescriptions; do not duplicate.
Dexamethasone	8 mg orally once daily	Days 2 and 3 after the oxaliplatin-containing dose Confirm the prescribed delayed prophylaxis and re-anchor it if treatment is delayed.
Metoclopramide	10 mg orally up to three times daily as required	Rescue only; duration to be completed by prescriber Review renal function, neurological contraindications and interactions.

Additional phase-specific support

- Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.
- Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- Avoid cold exposure as advised after oxaliplatin. Report persistent numbness or difficulty with buttons, walking or everyday tasks before the next dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Days 8, 22 and 36: fluorouracil/folinic acid only

Active medicines: Folinic acid + Fluorouracil

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before the fluorouracil-only treatment visit NCCP NCIS V8 low-risk default. Review QT risk, interactions, constipation and hepatic function; clinician may select an alternative.
Metoclopramide	10 mg orally up to three times daily as required	Rescue; prescriber confirms duration Check renal function, neurological contraindications and interactions.

Additional phase-specific support

- Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.
- Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.
- Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.
- Avoid cold exposure as advised after oxaliplatin. Do not suck ice chips during this regimen.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Mouth / eating

Fluorouracil

Report painful ulcers or difficulty eating and drinking.

2 Chest / heart

Fluorouracil

New chest pain needs urgent assessment, including during an infusion.

3 Bowel / hydration

Fluorouracil

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

4 Hands / feet

Fluorouracil

Report painful redness, cracking or peeling that affects daily activity.

5 Blood / infection

Chemotherapy

Fever, shivers or sudden illness need immediate oncology advice.

6 Energy / daily life

Treatment

Report fatigue that limits usual activity; ask for practical and nutrition support.

7 Nerves

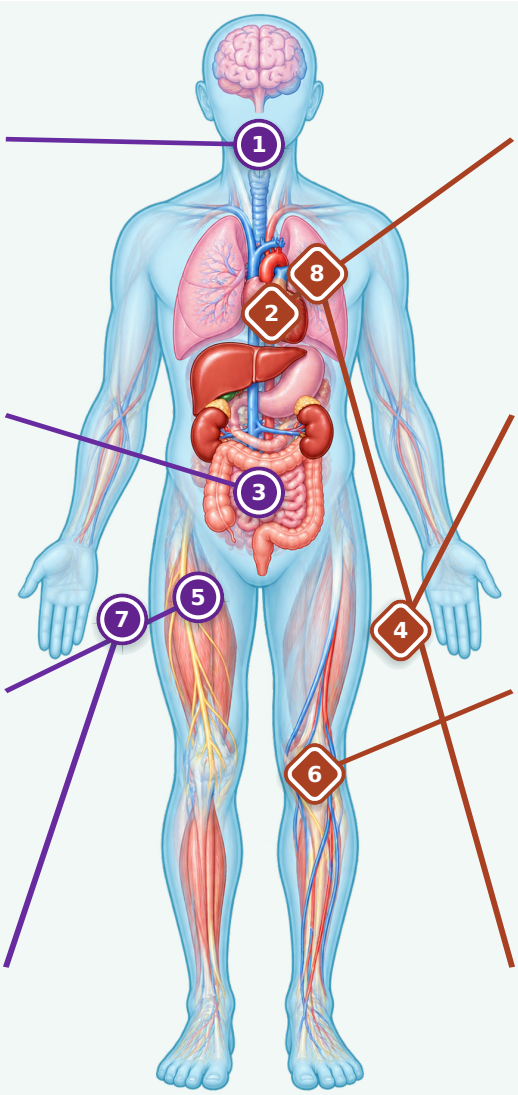
Oxaliplatin

Cold can trigger tingling. Report lasting numbness or difficulty with buttons or walking.

8 Throat / breathing

Oxaliplatin

Report throat tightness or breathing difficulty promptly; the team must assess the cause.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology immediately for fever, shivering, sudden illness, severe diarrhoea, inability to drink, very little urine or unusual bleeding. New severe chest pain, collapse or major breathing difficulty needs emergency help.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

SACTCHECK · CHAPTER 04

Sources and further information

FLOX: fluorouracil, folinic acid and oxaliplatin · National Cancer Control Programme (NCCP) 00486

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00486 v8 official regimen**
   https://healthservice.hse.ie/documents/6501/486_v8_FLOX.pdf
2. **NCCP NCIS antiemetics V8, Table 1; apply oral regimen guidance separately**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
3. **NSABP C-07 primary trial report**
   https://pubmed.ncbi.nlm.nih.gov/17470851/
4. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq